



Mental health advocates

Policy code	<i>SMHS: 162</i>
Original endorsement date	<i>October 2015</i>
Date of current endorsement	<i>August 2025</i>
Functional sub group	Mental health
Summary	The <i>Mental Health Act 2014</i> established the office of the Chief Mental Health Advocate, which administers the Mental Health Advocacy Service and engages other persons to be mental health advocates. These mental health advocates have specific powers to visit mental health services, speak with patients and other identified persons and make enquiries of mental health service staff.
Policy owner	Executive Director Mental Health
Contact	Carole Steiner
Review date	August 2028
National Safety and Quality Health Service (NSQHS) Standards	Standard 1: Clinical Governance Standard 2: Partnering with consumers
Status	<i>Active</i>

1. Background

1.1. About this document

The [Mental Health Act 2014](#) (MHA 2014) established the office of the Chief Mental Health Advocate, which administers the Mental Health Advocacy Service and engages other persons to be mental health advocates. These mental health advocates have specific powers to visit mental health services, speak with patients and other identified persons and make enquiries of mental health service staff.

Note: for the purposes of this policy, “mental health advocate” means a person engaged under section 350(1) of the [MHA 2014](#). Carers, close family members and other personal support persons are not mental health advocates. For information about the rights of these persons, refer to the South Metropolitan Health Service (SMHS) [Rights of Carers and other Personal Support Persons policy](#).

1.2. Key definitions

Term	Definition
Identified person	A person specified under the MHA 2014 s. 348 as having access to the Mental Health Advocacy Service. Refer to Section 4 - Who may be advocated for by mental health advocate.
Mental Health Service	Defined by the MHA 2014 s. 4 as: • A hospital only to the extent the hospital provides treatment or care to people who have or may have a mental illness this may include Emergency Departments, general wards, Intensive Care Units etc • Community mental health services
Personal support person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, nominated person, the parent or guardian of a child and any guardian or enduring guardian of an adult.

2. Policy Statement

Staff of a mental health service are required to comply with the [MHA 2014](#) in assisting mental health advocates to exercise their powers and functions under the Act.

The health impact upon Aboriginal people should be considered, and where relevant incorporated and appropriately addressed in the provision of advocacy services.

3. Roles and responsibilities

3.1. Executive

Each SMHS Mental Health Service Co director will ensure that managers and supervisors are aware of the requirement to comply with the [MHA 2014](#) in assisting mental health advocates to exercise their powers and functions under the Act.

3.2. Managers/Supervisors

Managers and supervisors should:

- Ensure the development of relevant local protocols and procedures as required.
- Ensure appropriate training and education is provided to staff in relation to mental health advocates
- Ensure that mental health advocates are provided with a site orientation in line with the SMHS Induction and Onboarding Policy and local procedures.
- Ensure that mental health advocates are advised of restricted documents that may not be shared with the identified person

3.3. Staff

Staff should:

- Comply with training requirements and proactively seek further training as required including Cultural Safety training that specifically addresses the mental health needs of Aboriginal people, to ensure culturally responsive and respectful service delivery.
- Comply with the [MHA 2014](#) in assisting mental health advocates to exercise their powers and functions under the Act.
- Ensure that Advocates are advised of restricted documents by obtaining approval from the Head of Service or treating Consultant Psychiatrist prior to complying with a request to access a document by a mental health advocate. Advice given to the mental health advocate must be documented in the identified persons medical record. [MHA 2014](#) s. 361.

4. Who may be advocated for by a mental health advocate?

The following persons, known as “identified persons”, may receive advocacy services from a mental health advocate:

- a person referred for an examination to be conducted by a psychiatrist to determine whether they need an involuntary treatment order
- a voluntary inpatient with respect to whom an order has been made for an assessment to determine if they need a voluntary treatment order
- a person detained for further examination by a psychiatrist
- an involuntary patient
- a person under a hospital order made under the [Criminal Law \(Mental Impairment\) Act 2023](#) (CLMI Act 2023)
- a detainee under the [CLMI Act 2023](#) detained at an authorised hospital
- a detainee under the [CLMI Act 2023](#) on release on condition that they undergo treatment
- a resident of a private psychiatric hostel; and

- a voluntary patient who is in a class of identified person, including a voluntary child seeking or receiving treatment

Refer to [MHA 2014](#) s. 348.

5. Request for a person to be contacted by an advocate

A request for a person to be contacted by a mental health advocate may be made by the person themselves, by their psychiatrist, a staff member or by a person who has sufficient interest in the identified person such as a personal support person.

The request may be made to the Chief Mental Health Advocate directly, or to the mental health service where the person is being detained or that is providing treatment or care to the identified person. If the request is made to the mental health service, it must ensure that the Chief Mental Health Advocate is notified as soon as practicable and in any event within 24 hours.

Refer to [MHA 2014](#) s. 356.

6. Functions of mental health advocates

An identified person must be visited or otherwise contacted by a mental health advocate:

- if referred or detained for examination by a psychiatrist, within three (3) days of the Chief Mental Health Advocate receiving a request
- if the person is an adult, within seven (7) days from the day on which the involuntary order is made
- if the person is a child, within 24 hours after the time that the involuntary order is made.

Refer to [MHA 2014](#) s. 352 and 357.

The functions of a mental health advocate are:

- visiting or otherwise contacting identified persons
- inquiring into or investigating any matter relating to the conditions of mental health services that is adversely affecting (or is likely to adversely affect) the health, safety or wellbeing of identified persons
- checking the extent to which identified persons have been informed of their rights, and the extent to which those rights have been observed
- seeking to resolve complaints by identified persons about their detention, treatment, or care
- referring issues to the appropriate persons or bodies for resolution, including to the Chief Mental Health Advocate
- assisting identified persons to protect and enforce their rights
- assisting identified persons to access legal services; and
- in consultation with medical practitioners and mental health practitioners, advocating for and facilitating access by identified persons to other services such as Aboriginal traditional healers.

Refer to [MHA 2014](#) s. 352.

In attempting to resolve any issue relating to the conditions of mental health services, the extent to which identified persons have been informed of their rights, or complaints by identified persons, a

mental health advocate may deal directly with the relevant staff members of the mental health service. Refer to [MHA 2014](#) s. 363.

7. Powers of mental health advocates

Mental health advocates have the power to:

- visit a mental health service at any time and for as long as the mental health advocate considers appropriate
- inspect any part of a mental health service
- see and speak with an identified person, unless the identified person objects to them doing so
- enquire about the admission or reception of an identified person, the referral of an identified person for examination, the detention of an identified person or the provision of treatment or care to an identified person
- require a staff member of a mental health service to answer questions, provide information and make available documentation
- inspect and take a copy of the whole or any part of the medical record of, or any document about, an identified person that is held by the mental health service unless the identified person objects to the mental health advocate doing so
- inspect and take a copy of the whole or any part of any document, or any document in a class of document held by the mental health service and prescribed by the [Mental Health Regulations 2015](#).

Identified persons who are involuntary patients are not required to provide consent. The [MHA 2014](#) s. 359(1)(c) and (f) set out that, in relation to access to the person and to their medical record and other documents about them, the Advocate can exercise their powers without consent as long as the identified person does not object.

Advocates will seek consent from a voluntary identified person, or if that voluntary identified person does not have capacity to consent, the person who is authorised by law to consent on their behalf [MHA 2014](#) s. 359(2).

Mental health advocates have the power to require a staff member of a mental health service to give reasonable assistance in the exercise of these powers. Refer to [MHA 2014](#) s. 362(1)(d).

8. Restricted documents

A person is not entitled to access a document relating to their treatment or care by a mental health service if:

- a psychiatrist reasonably believes disclosure would pose a significant risk of serious harm to, or to the health or safety of, the person or another person
- disclosure would reveal personal information about a third party
- disclosure would reveal confidential information that was obtained in confidence
- the person is or was a detainee under the [CLMI Act 2023](#), required to be detained at an authorised hospital and the document came into existence for the purposes of the [Prisons Act 1981](#).

Refer to [MHA 2014](#) s. 249.

If a person is not entitled to access documents relating to their treatment or care, then prior to making the document available to a mental health advocate a mental health service must advise the mental health advocate that the person has been refused access and the rationale that information must not be disclosed.

The mental health service must also record this advice on the person's medical record.

9. Compliance monitoring

9.1. Compliance monitoring

Each SMHS Mental Health service and executive director will ensure compliance with this policy.

9.2. Legislative obligations

Mental health service staff shall assist mental health advocates to exercise their powers and functions. Under the [MHA 2014](#), a staff member of a mental health service commits an offence if he/she:

- without reasonable excuse fails to answer a question asked by, or provide information requested by, a mental health advocate
- provides false or misleading answers or information to a mental health advocate
- provides false or misleading documents to a mental health advocate
- without reasonable excuse, fails to give reasonable assistance to a mental health advocate when required to do so; or obstructs or hinders a mental health advocate in the exercise of his/her powers or a person giving reasonable assistance to a mental health advocate.

Penalty: a fine of \$6000.

Refer to [MHA 2014](#) s. 362

10. Legislation

- [Criminal Law \(Mental Impairment\) Act 2023](#)
- [Mental Health Act 2014](#)
- [Mental Health Regulations 2015](#)
- [Prisons Act 1981](#)

11. Related Documents

11.1. SMHS policies

- [SMHS: 38 Rights of Carers and other Personal Support Persons Policy](#)
- [SMHS: 59 Induction and Onboarding Policy](#)

11.2. Other documents

- [National Safety Quality Health Service Standards 2nd Ed 2021](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)

11.3.Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 3.0	28 August 2025	15 August 2025	15 August 2028	August 2028	Triennial Review

6.6. Approval

Approval by	Executive Director Mental Health
Approval date	15 August 2025

This document can be made available in alternative formats on request.

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Compiled: South Metropolitan Health Service 2024

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